



Travel Tales

Mask and Vax Up for Summer Travel

by
Lew Toulmin

When you go on that neat summer driving vacation this summer, you're going to wear your seat belt, right? Of course. You are smart and good at risk assessment, and you know that driving without a seat belt can be deadly.

But, will you get another COVID shot before you go? Will you wear your N95 mask when you visit an indoor public place? Why not? Don't you know that you are four times more likely to die of COVID this summer than to have a deadly traffic accident?

Yes, the CDC has declared that the COVID emergency is over. But the fact is, COVID has become "endemic," meaning that it is "consistently present." I.e., it ain't going away. It's here to stay, like cancer, heart disease and gun violence. There are about 3,000 fatalities per month in the US from traffic accidents, but in March of this year there were over 12,000 deaths from COVID. And with the right mutation, COVID could spike again, even up to the 90,000 deaths reported in January 2021.

I have worked as a policy analyst for FEMA, USAID's Office of Foreign Disaster Assistance, DoD's National (emergency) Communications System, and various foreign government emergency management organizations. I know an emergency when I see one.

Consider how deadly COVID has been. In all of the major and minor wars the US has fought in since 1775, the total combat deaths were about 666,000 (including Confederate dead). Reported COVID deaths in the US now stand at about 1,100,000, almost twice as high!

Unfortunately, that "reported" number is almost certainly far too low. The nation's leading expert on COVID, the famous Dr. Anthony Fauci, estimated in July 2022 that the real number for COVID deaths was then not 1 million but rather "2 to 3 million," using the more reliable "excess deaths" estimation approach. Thus the current figure is likely close to or even over 3 million Americans killed by COVID. This is way more than four times all US combat deaths!

I guesstimate from current trends that COVID will kill about 400 Americans per day over the next year, or 146,000 souls. That is more than all the 140,414 US Army soldiers killed in combat in the five years of the Civil War. Some Republicans want a new civil war – they've already got it! (By the way, according to

two studies, Republicans are dying from COVID at twice the rate of Democrats, due to Republicans' vaccine and masking refusal.)

As in combat, the wounded from COVID are far more numerous. Recent studies have shown that about 6 percent of the US adult population, or about 16,000,000 people, have some form of "long COVID." Of these, US, UK and French studies have shown that long COVID usually lasts at least a year, and is often an indefinitely long disability. Sufferers experience memory loss, difficulty concentrating, sleep problems, loss of smell and taste, depression, anxiety, and even suicide. Getting support from employers, doctors and even friends and loved ones is often difficult. Obtaining COVID disability payments from Social Security is almost always a nightmare, taking up to two years of paperwork with a two-thirds rejection rate. My doctor says, "Long COVID can be worse than death."

But COVID is passé, right? It can't really affect your travel, correct? Well, think about this. If your summer trip takes you through, say, Dulles Airport, then you will be present for several hours in a facility that processes 66,000 people per day. Of those, over 200 will be infected with COVID, shedding those cute little viruses throughout the buildings, lingering in the air, ready to dive into your nose and lungs. Your airplane will be jam-packed, and those hotels, museums and concerts at your destination will also be crammed. Due to pent-up demand, about 42 percent of Americans are planning to take a summer vacation in 2023 involving a hotel stay or a flight, so over 140 million people are on the move – all of them breathing and coughing in your face!

What can you do to protect yourself? Wear a mask. And not one of those silly surgical masks that doctors wear on TV. Those just prevent the doc's spittle from getting into the patient's wounds. Wear an N95 or KN95 mask. After trying just about every mask on the market, I usually wear a KN95 four-layer adult "Well Before" mask, designed in Texas and manufactured in China. It is breathable and comfortable, with adjustable ear loops that don't hurt my ears. If I am going to be on a long plane ride where I want a mask with comfortable elastic straps around my head (not just my ears), I use a German Dräger X-plore 1759 mask, which is NIOSH certified at the N95 level.

And puLEEZE don't wear your mask below your nose or under your chin!

What else can you do? Get vaccinated. My wife and I (both over 65) have each been COVID vaccinated six times now. Even if you have had five shots including one "bi-valent" shot, by this summer your vaccination efficacy will likely be down to about 35 to 45 percent or less. Efficacy from each shot only lasts about 6 months, although the CDC is discussing only a once-a-year shot. The CDC has basically admitted that most Americans will resist getting a shot every six months, so the agency is doing the expedient thing, not the right thing for you and your travel safety.

Because of my extensive travel, over the past 30 years I have been vaccinated over 45 times, have successfully avoided all major diseases, and have had no ill effects from the shots. Inject THAT, anti-vaxxers!

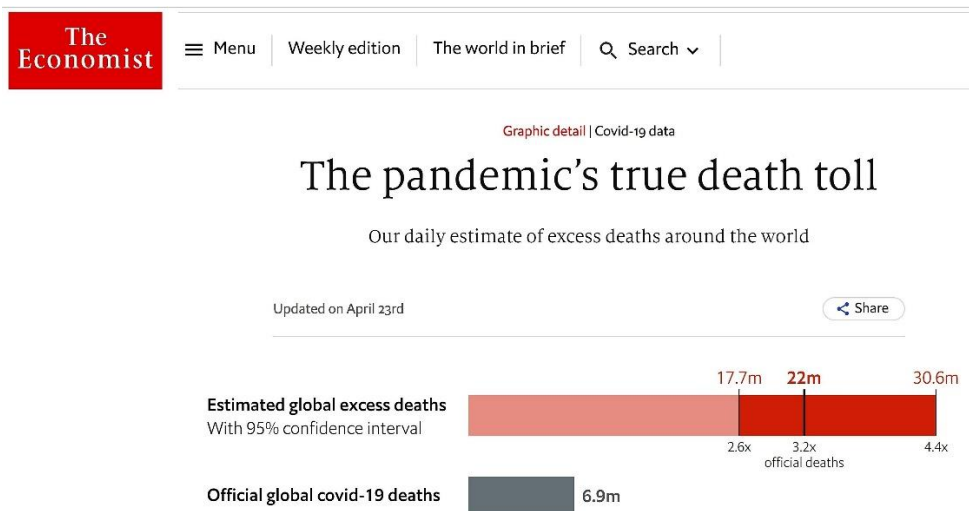
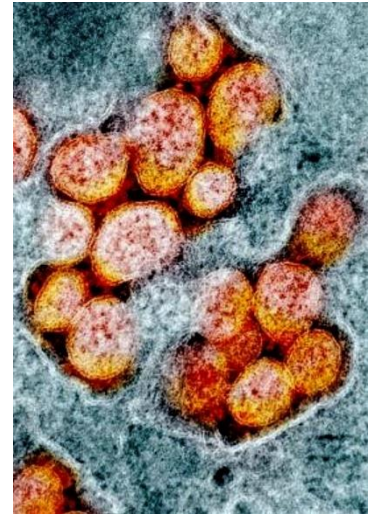
What else? Get your doctor to prescribe some Paxlovid to take with you, if you plan to travel to a location where that excellent treatment may be difficult to obtain. If you get COVID symptoms, take a test and call your doctor immediately to see if the Paxlovid is warranted -- quick treatment within five days is essential to reduce the chances of death or long COVID.

Now you are at least as safe as driving your car while wearing a seat belt. Have a great time!

Words in the story above: 1034

Images:

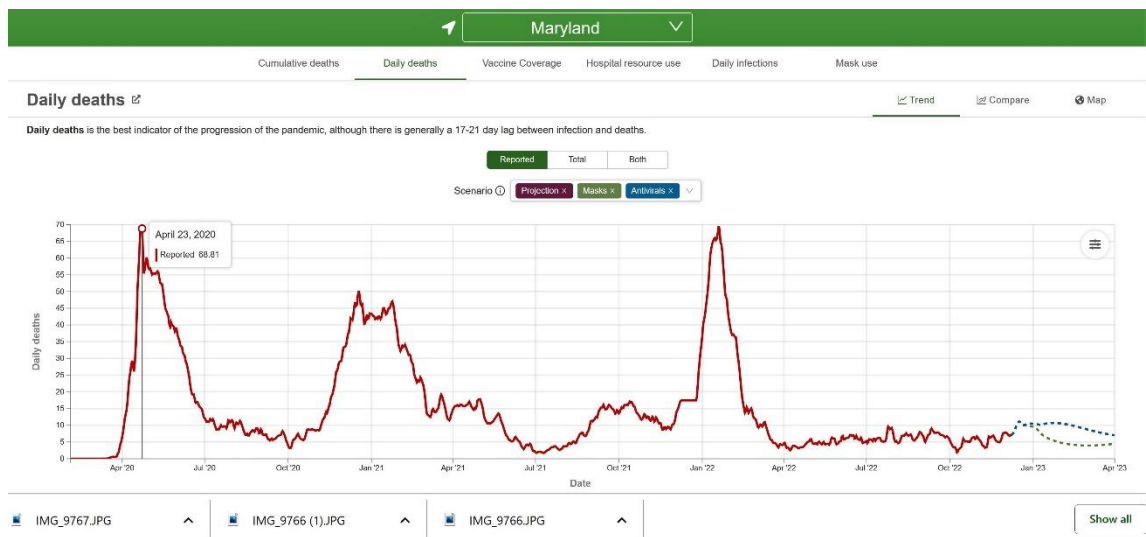
1. Actual COVID viruses, highly magnified (right). The nasty individual “virions” use a chemical key to lock in to human cells.
2. Sandra Lindsay (top pic), an intensive care nurse at the Long Island Jewish Medical Center in New York and an immigrant from Jamaica, receives the first COVID vaccine in history on December 14, 2020. She later served as Marshall of the New York City ticker tape parade honoring COVID health care workers, and was received by President Biden and the Prime Minister of Jamaica, Andrew Holness. The incredibly rapid development of the vaccine saved millions of lives around the world. She said, “I hope that anyone who is still vaccine hesitant can see how immunizations work to slow transmissions, hospitalizations and deaths from this unforgiving virus.”
3. The well-regarded *Economist* magazine states (chart below) that as of April 2023 official global COVID deaths are 6.9 million, while it estimates that actual deaths, using the “excess deaths” approach, at 17.7 to 30.6 million, or 3 to 5 times as many! “Excess deaths” estimates involve identifying the baseline level of deaths (before COVID), then calculating the excess increment attributable to COVID.



4. A face mask around your neck won't do a lot to stop COVID.
5. A survey of evangelicals done in August 2021 by the National Association of Evangelicals showed that 95 percent of evangelical pastors and leaders planned to get vaccinated, but almost none would urge their congregations to take the shot. Pastor Tony Spell (shown here below) of the Tabernacle Church in Baton Rouge, Louisiana has received national attention for his campaign against vaccination, claiming that "the virus has been a scam from the beginning." This, despite the fact that COVID has killed millions.



6. Paxlovid by Pfizer has been shown to be an effective treatment in reducing the effects of COVID, if taken immediately (within five days) after symptom onset.
7. A Dräger (a.k.a. Draeger) N95 X-plore 1759 mask with straps that go around the head is highly protective against COVID. Dräger, a German company, is one of the leading mask and self-contained breathing apparatus (SCBA) manufacturers in the world, having invented SCBAs for firefighters 80 years ago.
8. A Well Before KN95 mask, designed in America and made in China, has four layers of protection, is comfortable, and has adjustable ear loops. I wear it in situations where I want to be able to put it on and remove it quickly, such as going in to and then leaving a shop.
9. IHME (Institute for Health Metrics and Evaluation of the University of Washington) chart (below) for Maryland, showing the course of the entire COVID pandemic. The three major waves peaked at 70 deaths per day in Maryland on April 22, 2020, while current COVID deaths are running at about 9 per day. Total deaths in the state are reported at 16,883, but this is likely an underestimate by a factor of 1.5 to 5. About 89% of residents have had at least one vaccination. Current mask usage in Maryland is estimated by IHME at 13%, down from a peak of 85% in February 2021.



#end#

V1 column submitted May 1, 2023 to Montgomery Community Media